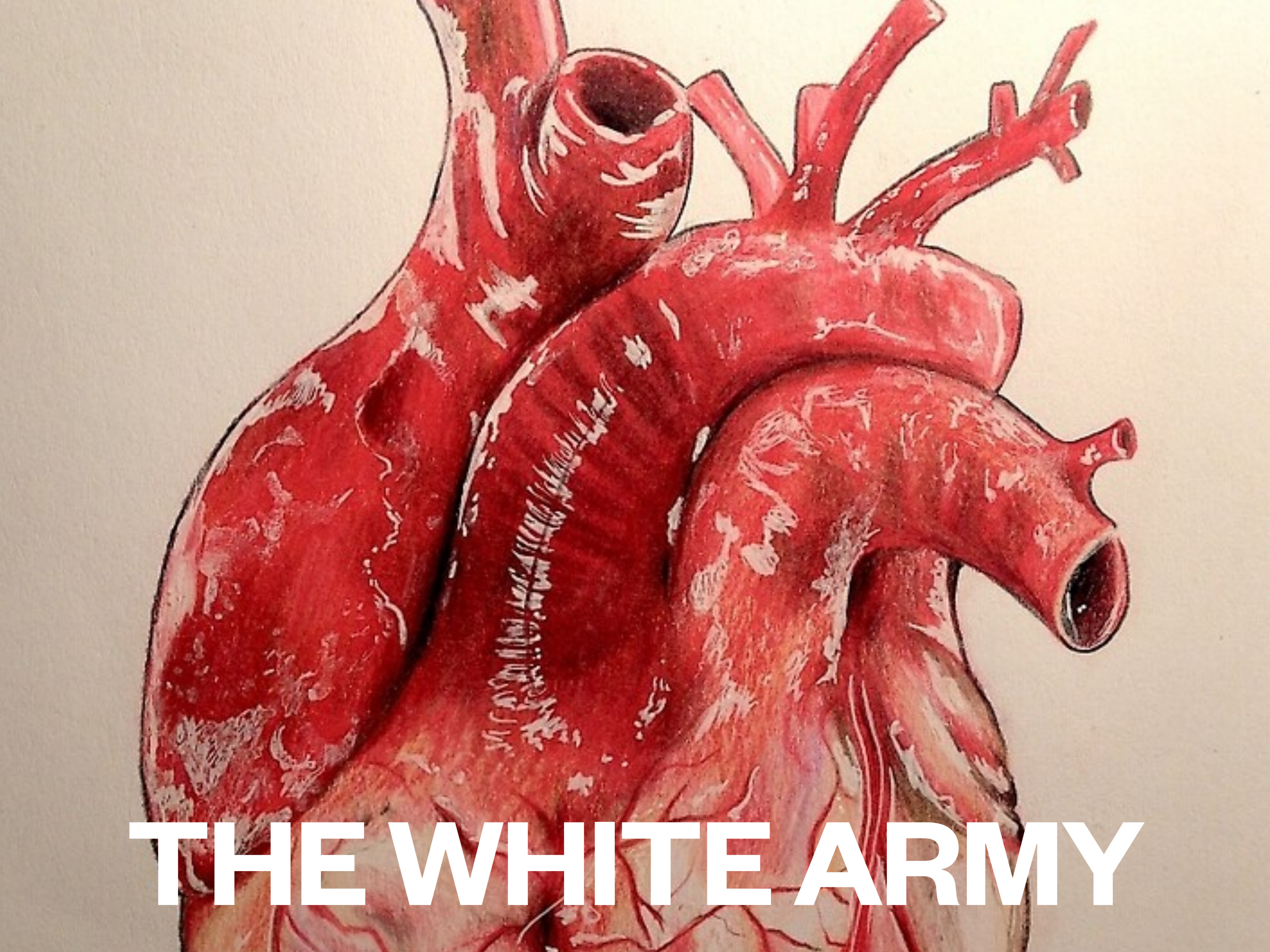


THE WHITE ARMY

MENTOR
Dr Archith Boloor

PRESENTED BY
NEHA
BMCRI

Patient particulars

- Name: ABC
- Age: 40 years
- Gender : female
- Address : kolar
- Class : lower middle class (modified kuppuswamy classification)
  Education : 12th class
- Marrital status : married

Chief complaints

- 1. Palpitations – 6 months
- 2. Breathlessness – 4 months
- 3. Cough – 3 weeks
- 4. Fever – 10days

History of presenting illness

- Patient was apparently normal 6 months ago when she started to experience unpleasant awareness of her heartbeat, insidious in onset, intermittently felt for around 10-15 minutes, thrice a week, exertional, irregular in rhythm, relieved on rest. Currently, she feels palpitations even at rest and sometimes during sleep since past 2-3 weeks. Not associated with chest pain, syncope. No post palpitational diuresis.

- Breathlessness since four months, insidious in onset. Initially she would become breathless when climbing 3 flights of stairs to her home, reduced on rest. It has gradually progressed such that since 2 months patient experiences breathlessness during sleep which would begin 2 hours after sleep, awakening her, reduced on sitting and taking deep breaths. Since 1 week patient is feeling breathless even at rest and increases immediately on lying supine on bed. Associated with fatigue which has been limiting her daily activities.
- not associated with wheeze.

- Cough since 2 weeks insidious in onset gradually progressive associated with expectoration of pink frothy sputum about 10 – 15 ml, non foul-smelling, increases on lying down, no seasonal variation.

- Fever since 10 days, continuous type, high grade associated with chills and rigors. Patient had presented to a private clinic and was prescribed medications following which fever reduced transiently but restarted the next day without any further improvement. During this period, patient had worsening of breathlessness, palpitations and cough.

- not associated with rash, joint pains, burning micturition, loose stools, abdominal pain, yellowish discolouration of skin/ eyes.
- no history of chestpain, syncope
- no history of weakness of limbs/difficult speech
- no history of lower limb swelling/abdominal distention
- no history of bluish discoloration of face/oral cavity
- no history of dysphagia/hoarseness of voice

Past history

- AT 16 years patient had an episode of fever with severe joint pain, at wrist which resolved followed by knee joint pain. Visited a local hospital and took medications for 10 days, was advised monthly injections but she never went back.
- Dental extraction procedure was done 2 weeks back.
- Not a k/c/o diabetes mellitus, hypertension,
- tuberculosis,asthma/thyroid disease.
- No h/o transient ischemic attacks
- No h/o similar complaints in past / hospitalizations.

Family history

- Nothing contributory

PERSONAL HISTORY

- Diet: mixed
- Appetite: normal
- Sleep: disturbed
- Bowel and bladder movements normal
- No known addictions

MENSTRUAL AND OBSTETRIC HISTORY

- Menarche at 12 years, regular and normal menstrual cycle since then.
- She gave birth to a child to first child 14 years back and second child 10 years back, it was uneventful normal vaginal delivery with no complications during or immediately after labour.

COURSE IN HOSPITAL

- 3 sets of blood samples taken from different venipuncture sites on day1 and then started on intravenous medications.
- Patient feels better with symptomatic improvement.

SUMMARY

- Middle aged female with no comorbidities with high grade fever since 10 days, worsening breathlessness (NYHA 2 to 4), irregular palpitations and cough with a past history of probable rheumatic fever.

- DIFFERENTIAL DIAGNOSIS
- 1. Valvular heart disease – mitral regurgitation > mitral stenosis
- 2. Congenital heart disease (ventricular septal defect)
- 3. Myxomatous degeneration of mitral valve (MVP)
- 4. ARDS secondary to pneumonia (pneumococcal)
- 5. Dilated cardiomyopathy

GENERAL PHYSICAL EXAMINATION

- The patient was examined on the 5th day of admission .
- A middle aged female, moderately built and nourishedwas conscious,cooperative and well oriented to time place and person. Patient was not having palpitations at the time of examination.
- Height : 155 cm Weight : 51 kg BMI : 21.2 kg/m2 VITALS:
- Pulse : 75bpm, normal volume and character,regular
- rhythm . No radio radial or radio femoral delay. All
- peripheral pulses were felt.
- BP : 110/70 MM Hg
- Resp. rate : 20 cycles/min.
- Temp : 97.7 F (TODAY) . On day of admission it was 101F
- JVP : 7 cm H2O . a and v waves were seen.

- Pallor +
- Icterus , cyanosis , clubbing, generalized lymphadenopathy and pedal edema were absent.
- Nails – Pale, Subungual splinter hemorrhages are seen.
- Small tender nodules on the skin near the fingers and toes (Osler's nodes) were seen on day 1, presently it was painless.
- Janeway's lesions were not seen.
- No features of arthralgia were noted.
- No markers of congenital heart disease , dyslipidaemia seen.
- No features of marfanoid habitus.



Inspection

- Precordium : No precordial bulge.
- Apex beat : Visible at left 6th ICS on Mid clavicular
- line(Shifted down and outwards)
- Trachea : Appears to be central.
- No engorged veins
- No visible pulsations were noted on the precordium, epigastric area or on the back.
- No scars were seen on the precordium.
- Chest wall was symmetrical.

Palpation

- Trachea : Central
- Apex beat is hyperdynamic, felt in 5th and 6th ICS on mid clavicular line.
- Parasternal heave : Absent
- Systolic thrill was palpable at the apex.

Auscultation

- Mitral Area : S1 and S2 were merged within the murmur. Not distinctively heard. LV S3 + Murmur : A pan systolic murmur of grade 4 best heard at the apex, with the diaphragm of the stethoscope when patient is in left lateral position and breath held in expiration. Radiation to the left axilla +. Murmur intensified on leg raising and decreased on standing.
- Tricuspid area : S1 and S2 +
- Aortic area : s1 and s2 +
- Pulmonary area : S1 and S2 +
- Carotid auscultation : No murmurs heard.

Other systems

- RS – NVBS heard bilaterally. Inspectory, palpatory , percussion and auscultatory findings were normal.
- Per abdomen : Soft, non tender abdomen. No organomegaly or ascites. Normal bowel sounds.
- CNS – No focal neurological deficits.

Summary

- Middle aged female with no comorbidities with high grade fever since 10 days, worsening breathlessness (NYHA 2 to 4), irregular palpitations and cough with a past history of probable rheumatic fever.
- On examination : She had pallor, subungual splinter haemorrhages, osler’s nodes, Hyperdynamic apex shifted outwards and downwards and a Pan Systolic murmur best heard at the apex on expiration.

Provisional diagnosis

- Acquired valvular heart disease in the form of Chronic Mitral Regurgitation probably of rheumatic Aetiology, with possible paroxysmal atrial fibrillation with features of Left ventricular dysfunction and features suggestive of Acute Infective Endocarditis.

Investigation

- Blood culture for organisms.
- Complete Blood Count.
- Transthoracic Echocardiography
- Cardiac doppler studies.
- LFT, RFT
- Chest Xray if needed.
- Direct Ophthalmoscopy (Fundoscopy).

Treatment

- If patient is hemodynamically unstable : Start empirical antibiotics after collecting required blood culture samples.
- Hemodynamically stable / subacute feature : Empirical therapy can be with held until awaiting the culture reports. Treatment for fever.
- Organism specific antibiotic therapy (Containing penicillin, cephalosporins, vancomycin)
- Treatment of heart failure : Diuretics
- Atrial fibrillation : Digoxin , Anticoagulants (guided by CHAD2 – VASc scoring).



Thank you